

J&J advances JNJ-4804 (guselkumab+golimumab) into phase 3 for UC & CD



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Published 5/13/2026

Summary

J&J has posted two phase 3 studies (DUET ENCORE-UC and DUET ENCORE-CD) for JNJ-78934804 / JNJ-4804, a fixed-dose combination of guselkumab (IL-23) and golimumab (anti-TNF), positioning it as the first co-antibody therapy in Crohn's disease (CD) and ulcerative colitis (UC). The studies directly compare the combination therapy against guselkumab monotherapy in moderately to severely active IBD (UC & CD) and follow mixed phase 2b results in which overall primary endpoints were missed, but efficacy signals were observed in highly refractory patient subgroups.

Why it Matters: J&J's decision to advance JNJ-4804 into phase 3 reflects high unmet medical needs of patients with IBD due to the efficacy ceiling of current therapies. While the DUET phase 2b program failed to demonstrate superiority over guselkumab on primary endpoints in the overall population, both studies showed a consistent efficacy signal in highly refractory patients (inadequate responders to ≥ 2 advanced therapy classes), with a safety profile generally in line with the component monotherapies. However, the phase 3 study posting didn't disclose whether inadequate responses to two or more advanced therapies are required.

Key Highlights

- J&J has posted two phase 3 studies (DUET ENCORE-CD & DUET ENCORE-UC) for JNJ-78934804 / JNJ-4804 (guselkumab + golimumab), positioning it as the first co-antibody therapy in CD and UC
- DUET ENCORE-CD (NCT07577843) in moderately to severely active CD and DUET ENCORE-UC (NCT07577856) in moderately to severely active UC. Both are randomized, double-blind, multicenter trials comparing JNJ-4804 head-to-head against guselkumab monotherapy, with planned enrollment of ~460 (CD) and ~644 (UC) participants
- Patient inclusion criteria include "*inadequate initial response, loss of response, or intolerance to previously approved systemic therapies*", but it's not clear how many advanced therapies patients must experience
- DUET ENCORE-CD primary completion is estimated for June 2028, and DUET ENCORE-UC primary completion is estimated for August 2028 placing first topline reads roughly 2-2.5 years out. Full study completion (including the long-term extension phase) is estimated for July 2030 (CD) and October 2030 (UC)
- Both ENCORE studies removed placebo and golimumab, leaving guselkumab monotherapy as the only comparator. Since guselkumab is already approved in both UC and CD, JNJ-4804 need to demonstrate clinically meaningful superiority over an effective standard rather than placebo-level differentiation

- ENCORE study postings do not yet specify the registrational dose, creating a key open question: if J&J escalates to approved-equivalent doses in phase 3, the delta may expand meaningfully OR if the phase 2b high-dose is retained (likely constrained by single-injection volume requirements for the fixed-dose SC), phase 3 is largely a powered re-test of the phase 2b hypothesis
- Phase 2b trials of JNJ-4804 for Crohn's disease ([DUET-CD](#)) and ulcerative colitis ([DUET-UC](#)) presented at DDW 2026, found that the combination was not superior to guselkumab alone in overall study populations. However, subgroup analysis revealed the combination was significantly more effective in patients who had inadequate responses to two or more advanced therapy classes (BMF Presentation [LINK](#))
- Earlier, last month Royalty Pharma and J&J have announced a \$500M R&D co-funding agreement (2026–2027) to advance development of JNJ-4804 ([link to VITAL](#))

Details

- **DUET ENCORE-UC ([NCT07577856](#))**
 - A Phase 3, Randomized, Double-blind, and Active-controlled, Multi-center Study to Evaluate the Efficacy and Safety of JNJ-4804 in Participants With Moderately to Severely Active Ulcerative Colitis
 - Sponsor: Janssen Research & Development, LLC (Johnson & Johnson); Status: Not yet recruiting
 - Design: Phase 3, randomized, double-blind, active-controlled (guselkumab), parallel-group, multicenter; treatment purpose; participant- and investigator-masked
 - Enrollment: ~644 participants (estimated)
 - Arms / Dosing:
 - JNJ-4804 (Experimental): SC induction at Weeks 0, 4, 8 → SC maintenance Q4W from Week 12; rescue criteria → re-induction at Weeks 16, 20, 24 → maintenance Q4W from Week 28; eligible Week-48 completers enter LTE
 - Guselkumab / TREMFYA (Active Comparator): SC induction at Weeks 0, 4, 8 → SC maintenance Q4W from Week 12; rescue criteria → switch to JNJ-4804 induction at Weeks 16, 20, 24 → maintenance Q4W from Week 28; LTE for Week-48 completers
 - Route: Subcutaneous (SC)
 - Endpoints:
 - Primary endpoint (Baseline → Week 48): percentage of participants in clinical remission (Mayo stool frequency subscore 0/1, rectal bleeding subscore 0, and endoscopy subscore 0/1)
 - Secondary endpoints: endoscopic improvement (endoscopy subscore 0/1) at Wk 48; corticosteroid-free (90-day) clinical remission at Wk 48; combined histologic remission (Geboes) + endoscopic improvement at Wk 48; fatigue response on PROMIS Fatigue SF-7a (≥ 7 -pt reduction) at Wk 48; sustained Mayo SF ≤ 1 and RB=0 at Wks 12, 24, and 48; abdominal pain remission at Wk 48; IBDQ response (≥ 16 -pt) at Wk 48; PROMIS-29 MCS response (≥ 7 -pt) at Wk 48; clinical remission at Wk 12; endoscopic improvement at Wk 12; Mayo SF/RB response at Wk 2; AEs/SAEs through ~3 years
 - Key inclusion: Adults ≥ 18 yrs; UC diagnosed ≥ 12 weeks before screening (endoscopic + histopathology evidence); moderately to severely active UC defined as baseline modified Mayo score 5-9 (inclusive) using centrally-read Mayo endoscopy subscore; endoscopy subscore ≥ 2 on central review; inadequate initial response, loss of response, or intolerance to previously approved systemic therapies
 - Key exclusion: Isolated proctitis (UC limited to rectum or < 20 cm from anal verge); indeterminate, microscopic, ischemic colitis, Crohn's colitis, or findings suggestive of CD; history of or ongoing chronic/recurrent infectious disease; prior inadequate response, loss of

response, allergy, hypersensitivity, or intolerance to guselkumab or golimumab; pregnancy, breastfeeding, planning pregnancy, or planning to father a child during study or within 6 months after last dose

- Locations: Not yet disclosed (study not yet recruiting); multicenter (global; EU registered)
- Timeline: Estimated start May'26; primary completion Aug'28; study completion Oct'30

- **DUET ENCORE-CD (NCT07577843)**

- Phase 3, Randomized, Double-blind, and Active-controlled Multicenter Study to Evaluate the Efficacy and Safety of JNJ-4804 in Participants With Moderately to Severely Active Crohn's Disease
- Sponsor: Janssen Research & Development, LLC (Johnson & Johnson); Status: Not yet recruiting
- Design: Phase 3, randomized, double-blind, active-controlled (guselkumab), parallel-group, multicenter; treatment purpose; participant- and investigator-masked
- Enrollment: ~460 participants (estimated)
- Arms / Dosing:
 - JNJ-4804 (Experimental): SC induction at Weeks 0, 4, 8 → SC maintenance Q4W from Week 12; participants meeting rescue criteria receive re-induction at Weeks 16, 20, 24 → maintenance Q4W from Week 28; eligible Week-48 completers enter long-term extension (LTE)
 - Guselkumab / TREMFYA (Active Comparator): SC induction at Weeks 0, 4, 8 → SC maintenance Q4W from Week 12; rescue criteria → switch to JNJ-4804 induction at Weeks 16, 20, 24 → maintenance Q4W from Week 28; LTE for Week-48 completers
- Route: Subcutaneous (SC)
- Endpoints:
 - Primary endpoints (Baseline → Week 48) — co-primary: percentage of participants with clinical remission (CDAI <150); percentage with endoscopic remission (SES-CD ≤4 with ≥2-point reduction from baseline and no individual subscore >1)
 - Secondary endpoints: deep remission (clinical + endoscopic) at Wk 48; corticosteroid-free (90-day) clinical remission at Wk 48; corticosteroid-free (90-day) PRO-2 remission at Wk 48; sustained clinical remission (Wks 12 and 48); histologic-endoscopic remission at Wk 48 (Robarts Histopathology Index); IBDQ response (≥16-pt improvement) at Wk 48; PROMIS-29 MCS response (≥7-pt) at Wk 48; clinical remission at Wk 12; endoscopic response at Wk 12; AEs/SAEs through ~3 years
- Key inclusion: Adults ≥18 yrs; CD or fistulizing CD diagnosed ≥12 weeks before screening (endoscopic + histopathology evidence); moderately to severely active CD (baseline CDAI ≥220 to ≤450) with mean daily SF ≥4.0 or mean daily AP ≥2.0; moderately to severely active ileal/colonic CD on central-read screening ileocolonoscopy per SES-CD; inadequate initial response, loss of response, or intolerance to previously approved systemic therapies
- Key exclusion: Indeterminate, microscopic, ischemic colitis, UC or findings suggestive of UC; CD complications (symptomatic strictures/stenoses or any manifestation likely to require intestinal surgery on-study); draining stoma or ostomy; short bowel syndrome, missing >2 of 5 ileocolonic segments, or other condition that would confound CDAI-based efficacy assessment; current or suspected abscess
- Locations: Not yet disclosed (study not yet recruiting); multicenter (global)
- Timeline: Estimated start May'26; primary completion Jun'28; study completion Jul'30

- **Observations/Comments:**

- Disclosures were presented at DDW 2026 - phase 2b DUET UC ([PowerPoint Presentation](#)) and phase 2b DUET CD ([PowerPoint Presentation](#))
- BMF Presentation ([LINK](#)) and join the BMF discussion ([MS Teams](#))

Sources

DUET ENCORE-UC ([NCT07577856](#)); DUET ENCORE-CD ([NCT07577843](#))

LRL Competitive Intelligence Team

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